DIAGNOSTIC ARTIFACT

NHS Primary Care — Structural Diagnosis of Recurring Continuity Failure

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Classification: Public diagnostic exemplar — derived from a full internal diagnostic artifact and based entirely on open sources

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Evidentiary standard: This artifact distinguishes directly evidenced facts [1][2], convergent public symptom patterns [4][5], consequence evidence [3], workforce and collapse node evidence [4], ghost structure and patient experience evidence [5], and structural inference [6]. The governing diagnosis is a high-confidence structural inference from convergent public evidence — not a claim about internal intent or individual failure. Evidence tier references appear inline as [1]–[6]; source classes are defined in Section 15.

Judgment boundary: This artifact makes no claim about clinical negligence, individual clinician failure, institutional bad faith, or intentional misconduct by any NHS clinician, practice, trust, or vendor. It is a structural reading of public evidence. The failures described are architectural, not personal — consistent with decades of fragmented IT procurement and governance, not with malice or incompetence on the part of individuals. No inference is made about internal unpublished system logic, undisclosed contractual arrangements, or adjudicated legal liability beyond what public sources directly support. All coroner and legal references are used as pattern evidence, not as adjudicated findings of liability against any named party.

SECTION 0 — WHAT THIS DOCUMENT IS

This is a structural diagnosis from public evidence.

It is not a legal brief, a policy manifesto, a technology procurement recommendation, a clinical guideline, or a redesign specification. It identifies the governing primitive, the first structural break, the burden-carrier layer, the function-collapse condition, and the high-level dependency order for correction — derived entirely from publicly observable system behaviour.

This public version preserves the core diagnostic spine to demonstrate method, but does not disclose the complete replacement specification. The full internal redesign artifact specifies the target-state architecture, transition sequence, and implementation logic. That is a separate object.

SECTION 1 — EXECUTIVE SUMMARY

NHS primary care has a recurring continuity failure. Patients repeat their medical history at every appointment. Discharge summaries arrive late, incomplete, or not at all. The same unresolved symptoms present across multiple consultations with no clinical alerting mechanism. Successive interventions have not resolved the core failure. They have moved it.

The governing diagnosis: the NHS has externalised its operational memory function onto the patient by architectural default — while fusing sensing, interpretation, authority, and memory reconstruction into a single ten-minute GP consultation operating without structural support for any of them.

This failure is produced by a single structural condition: operational memory is not available at the point of clinical consequence. The patient repeats their history because the system cannot retrieve it. Because memory is absent, the GP must simultaneously sense, interpret, decide, and reconstruct prior state in a ten-minute window under full clinical consequence — with no structural support for any of these functions.

This produces predictable downstream effects: diagnostic delay, undetected repeat presentations, preventable harm, and rising negligence exposure. Coroners issued 36 Prevention of Future Deaths warnings in a single year for information sharing failures that contributed to patient deaths [3]. Clinical negligence costs the NHS £2.8 billion annually, with failure or delay in diagnosis accounting for 43.5% of all GP indemnity notifications [3]. Over 40% of GPs doubt they will still be practising in five years [4].

The system has constructed audit visibility but not operational continuity. Every prior fix has addressed this at the wrong layer. NPfIT, the Summary Care Record, the Shared Care Record, the NHS App, access reforms, ARRS workforce expansion, and the new contractual continuity requirement have each improved documentation density or audit visibility without restoring operational memory to the point of care [2]. HSSIB confirmed in 2023 that there is no GP contract requirement to deliver continuity of care and no GP IT standard requiring relevant information to be surfaced when patients return with unresolved symptoms [2].

One-line verdict: the NHS has built the audit layer and left the operational layer with the patient. The GP consultation is the collapse node. Correct the primitive, or every future programme will relocate the same failure.

SECTION 2 — ENGAGEMENT SCOPE

System under analysis
NHS primary care continuity failure at the GP-led front door and across the GP → referral → secondary care → discharge → GP return pathway.

Included in scope: GP consultation (first contact and repeat presentation); referral generation and receipt; specialist handoff; discharge back to GP; record retrieval at the point of consultation; patient and carer continuity burden; NHS information architecture (Summary Care Record, Shared Care Record, GP Connect, NHS App).

Excluded unless directly relevant: broad NHS finance reform; hospital management generally; unrelated digital health policy; international health systems (referenced only as structural comparators after the governing diagnosis is formed).

Evidence basis
This diagnosis is based entirely on publicly available materials [1][2][3][4][5][6]. Primary sources: NHS England official materials; HSSIB investigation reports (2023, 2025 ×2); NHS Resolution annual data; NAO digital transformation analysis; RCGP continuity and workforce reports; BMA pressures analysis; Healthwatch England national survey (n=1,800, March 2025); Coroner Prevention of Future Deaths reports (2024); peer-reviewed literature from BJGP, BMJ, Lancet, and PubMed.

Known fixed constraints: the GP contractor model; demand-driven appointment availability; the 10-minute standard consultation unit; the multi-vendor EHR landscape; and the governance, privacy, and commercial environment surrounding NHS health data.

SECTION 3 — FAILURE PATTERN AS PRESENTED

The following is synthesised from NHS England official materials, RCGP workforce and continuity reports, BMA pressures analysis, HSSIB investigation findings, and patient experience surveys — April 2026. It represents how NHS leadership, commissioners, and the GP workforce describe the failure before structural translation.

"GPs are under unsustainable workload pressure. They do not have enough time in appointments. They are spending a significant proportion of their working day on administrative tasks rather than clinical care. The workforce is shrinking and people are leaving the profession." [4]

"Patients are struggling to access their GP. When they do get an appointment, they often cannot see a doctor they know. Continuity of care has declined significantly over the past decade." [1][5]

"Records are fragmented. GPs do not always have the information they need — especially following a hospital discharge. Discharge summaries are often delayed or incomplete. Different parts of the system use different IT, and it does not reliably communicate across organisations." [2]

"We have invested significantly in digital infrastructure — the NHS App, the Shared Care Record, GP Connect, and the Summary Care Record. Progress has been made but the problem persists." [1]

"Patients with long-term conditions are having multiple appointments without their care being properly coordinated. The same issues keep returning. Referrals are delayed. Diagnoses are missed." [2][3]

"We know continuity of care produces better outcomes. We have introduced a contractual requirement for it in the 2024/25 contract. But it is difficult to deliver when the workforce is stretched, practices are large, and the digital infrastructure does not yet fully support it." [1][2]

These are not isolated complaints. They describe recurring friction across the full patient lifecycle — intake, consultation, referral, discharge, and return [1][2][4][5]. The pattern is identical across record access, clinical handoff, workforce sustainability, and patient experience. That is the first structural signal that this is not a capacity problem or a digital investment problem in isolation.

SECTION 4 — EARLY STRUCTURAL SIGNALS

Four structural signals are visible from the public record before the formal diagnosis is applied:

4.1 The system promises continuity but has no structural mechanism to guarantee it. HSSIB's 2023 investigation found no GP contract requirement for practices to deliver continuity of care, no standard national framework for doing so, and no GP IT requirement to surface relevant information when patients repeatedly present with unresolved symptoms [2]. The 2024/25 contract introduced the first contractual continuity requirement since 1948 — at the moment when patient-reported continuity was at its historical nadir [1][2]. Continuity is treated as a desirable outcome rather than a structural requirement with a defined delivery mechanism.

4.2 Every correction has addressed the documentation layer, not the retrieval layer. NPfIT created records without interoperability [6]. The SCR created minimal cross-organisational access without the clinical picture required for judgment [1]. The Shared Care Record created professional read-access without real-time retrieval [1][2]. The NHS App created conditional patient access to a slice of the GP record from November 2023 [1]. Each intervention improved trace — the institution's ability to record what happened — without improving retrieval: the clinician's ability to access what they need at the moment a decision must be made [2][6].

4.3 The patient is formally carrying the memory function.
NHS England's health and care passport guidance actively encourages patients to maintain a concise personal summary of the information professionals need to care for them safely [1][5]. Healthwatch England's 2025 national survey found 26% of patients with record errors must repeat their full medical history at every appointment [5]. This is not patient empowerment. It is the institution formally acknowledging that it cannot surface its own operational memory — and asking the patient to carry it instead.

4.4 The consequence layer is patterned, not random.
Coroners issued 36 Prevention of Future Deaths warnings in 2024 specifically about information sharing failures causing patient deaths [3]. NHS Resolution receives clinical negligence claims at one every 40 minutes [3]. A Lancet study found lack of GP continuity associated with a 30% increase in emergency hospital admissions [6]. These consequences cluster around the same structural origin. They are not coincidental.

SECTION 5 — SYSTEM BOUNDARY MAP

The system operates as a distributed record field with no central continuity node.

Records exist across multiple organisational silos. Access is conditional, delayed, and request-based rather than native. Continuity is reconstructed at the consultation boundary by whoever is present. This produces a system shape where memory is upstream but inaccessible, decisions are downstream and immediate, and the gap between them is bridged manually — by the patient, the carer, or the GP's personal recollection — at the point of consequence.

Primary actors:

Actor	Role	Position in system
Patient	Carries own continuity history; repeats at each encounter	De facto memory carrier
Carer / family member	Maintains medication logs, appointment histories, health diaries	Secondary memory carrier
GP (known / relational)	Clinical judgment with accumulated patient knowledge	Collapse node — relational continuity available
GP (pooled / unknown)	Clinical judgment without prior patient knowledge	Collapse node — memory reconstruction required

Receptionist / care navigator	Informal triage, appointment routing, complexity flagging	Informal sensing and continuity brokering layer
NHS 111	Remote triage and urgent referral	Parallel sensing node — limited record access
Secondary care / specialist	Specialist assessment and treatment	Separate record silo
Discharge summary	Information transfer from secondary to primary care	Handoff mechanism — routinely delayed or incomplete [2]
Summary Care Record	Medications, allergies, adverse reactions — national minimal record	Audit-grade memory layer [1]
Shared Care Record / ConCR	Cross-organisational professional read-access	Partial operational layer — programme closed March 2026 [1]
NHS App	Patient-facing GP record access	Partial visibility layer — GP summary from November 2023 only [1]

Key interfaces and where failure concentrates:
The first contact interface routes patients to appointments without clinical record access [4][5]. The consultation interface opens onto a fragmented record — GP notes, SCR summary, and whatever secondary care has uploaded — but not necessarily the full operational picture [2]. The discharge interface generates a summary document targeted for 24-hour delivery that routinely arrives late, incomplete, or not at all [2]. The return consultation interface opens onto whatever was present before the patient arrived — which may not include the discharge summary the patient assumes the GP already has [2][5].

Decision points where structural failure concentrates:

- Whether usable clinical history is present when the GP opens a returning patient's record [2][5]
- Whether a repeat unresolved presentation triggers any clinical alerting — currently: no IT standard [2]
- Whether a discharge summary has arrived and been uploaded before the follow-up appointment [2]
- Whether the consulting clinician has any prior knowledge of the patient [4][5]

Ownership nodes:

- NHS England: national standards, contract requirements, programme commissioning [1]

- NHS Digital / NHSE Tech: EHR standards, GP IT procurement, interoperability architecture [1][2]
- GP practices: individual record custodians; no cross-practice data flow by default [1][2]
- Patients: de facto operational continuity carriers — unrecognised, uncompensated [1][5]
- EHR vendors (EMIS, TPP): API control; structural interoperability constraint [2][6]

SECTION 6 — WORKFLOW AND PIPELINE TRACE

The documented operating sequence:

Present symptom → Contact practice → Route to appointment → Open record → Reconstruct history → Sense, interpret, decide → Document → [If referral] → Secondary episode → Discharge summary → GP receipt → Patient returns → Repeat

1. Patient develops symptom; contacts practice by phone or online system [1][4]
2. Receptionist or care navigator routes to available appointment — not necessarily the patient's usual GP [1][4]
3. GP opens consultation; record displays GP notes, SCR summary, any uploaded correspondence [1][2]
4. GP reconstructs clinical history from partial record and patient verbal account — the patient is the primary continuity source [2][5]
5. GP senses presenting symptoms, interprets against reconstructed history, makes consequential clinical decision [2][4]
6. GP documents encounter — written for audit defensibility as much as clinical handoff [4][6]
7. If referral generated: secondary care episode occurs in separate EHR with no automatic GP record update [2]
8. Secondary care produces discharge summary — standard target: 24 hours; actual: routinely delayed, incomplete, or not sent [2]
9. Summary received at GP practice; reviewed and uploaded by administrative staff — not automatically available in consultation view [2]
10. Patient returns for follow-up; cycle repeats from step 3 with no guarantee the discharge summary is present [2][5]

Critical structural observations:

There is no step where operational memory is structurally guaranteed to be present before the clinical decision must be made [2][6]. The workflow assumes the clinician will reconstruct what the system cannot retrieve.

The discharge summary arrives after the patient [2]. HSSIB's 2025 investigation confirmed that discharge summaries routinely arrive late, incomplete, or not at all — and in the gap, the GP relies entirely on the patient or carer's verbal account. HSSIB documented cases where

summaries had not been sent for approximately a year, producing near misses and confirmed patient harm.

The pooled list model eliminates relational continuity by structural default [1][4]. A patient presenting five times in six months — the pattern most likely to indicate an unresolved diagnostic need — is statistically unlikely to have seen the same GP twice.

There is no alerting mechanism for repeat unresolved presentations [2]. The same symptom coded differently across multiple consultations does not trigger a clinical flag.

SECTION 7 — FUNCTIONAL DIAGNOSIS

Applying the Four-Function Law [6]: institutions fail under scale when sensing, interpretation, authority, and memory remain fused or misaligned at the point of consequence.

Function	Formal location	Actual location in execution	Condition	Consequence
Sensing	NHS 111, digital triage, practice receptionists	GP in consultation — absorbing signals from patient verbal account, record fragments, and presenting symptoms	**Misaligned** — sensing occurs at the collapse node after the pre-consultation opportunity is gone	Signal quality depends on patient recall and record availability. Weak signals — the unresolved pattern across five encounters with different GPs — are structurally invisible [2][5]
Interpretation	Clinical frameworks, NICE guidelines, specialist input	GP in 10-minute consultation, reconstructing history while forming clinical judgment	**Fused with authority** — interpretation and decision occur simultaneously with no structural separation	Judgment is formed from incomplete information. HSSIB confirmed: where continuity systems exist, GPs have more time to process information [2][4]

Authority	GP as registered clinician	GP in 10-minute consultation — decides, prescribes, refers, or discharges	**Fused with sensing and interpretation** — no structural separation between signal reception and consequential action	43.5% of GP indemnity notifications cite failure or delay in diagnosis [3]. This is not primarily a clinician failure. It is an authority node receiving degraded inputs.
Memory	NHS EHR, SCR, GP record, Shared Care Record	Patient (verbal history at each encounter); carer (medication logs, appointment diaries); GP (accumulated knowledge for known patients only)	**Structurally absent at point of need** — externalised to patient without design, recognition, or compensation	The memory function is performed by the person least structurally equipped to carry it. When that person cannot, the system has no fallback [2][3][5]

The critical fusion: sensing, interpretation, and authority are collapsed into the GP consultation. The 10-minute time unit eliminates any structural space in which they could be separated [4][6].

The critical absence: operational memory is not reliably present at the point of clinical consequence. The SCR, the NHS App record summary, and the Shared Care Record are audit-grade: they improve defensibility without restoring the clinical picture required for judgment [1][2].

The critical inversion: the NHS has instrumented itself heavily for audit and compliance while the operational continuity function has migrated outward [1][2][6]. Every documentation investment has deepened this inversion.

SECTION 8 — STRUCTURAL FAILURE POINT

The first structural break: the system is not designed to deliver continuity at the point where decisions are made. Continuity exists only if carried by a human.

This is not the delayed discharge summary — that is a downstream consequence. This is not the ten-minute appointment — that is the structural container in which the consequences of the

break are forced under compression. This is not GP burnout or workforce shortage — each is a further downstream effect.

The first break is earlier and less visible. It accumulated through decades of fragmented IT procurement, organisational record ownership, and digital reform that prioritised documentation over retrieval [2][6]. It has become the governing structural fact of NHS primary care.

The three capability modes — how the system handles each:

Mode	Description	How the NHS handles it
Execution reliability	Consistent appointment delivery, prescription processing, routine referral	Actively managed and measured. This is what access and throughput metrics capture. [1][4]
Diagnostic capability	Pattern recognition across encounters, escalation of unresolved presentations, detection of weak signals over time	Present for patients with relational continuity to a known GP. Structurally absent across the pooled list model and care transitions. [2][4]
Continuity retrieval	Surfacing usable clinical history at the point of consequential decision	No contract requirement, no IT standard, no architectural guarantee. [2][6]

The third capability — the one that would connect five appointments, surface the medication prescribed in A&E that the GP does not know about, and flag the symptom coded differently three times — is absent not because the data does not exist. It is absent because no structural mechanism surfaces it at the moment it is needed.

Evidence of the break:

HSSIB (2023) found no GP contract requirement for continuity and no IT standard to surface repeat unresolved presentations [2]. HSSIB (2025) confirmed that when discharge information has not arrived, the GP relies on the patient or carer's verbal account to make care decisions [2]. Healthwatch England's 2025 national survey found 26% of patients with record errors repeat their full history at every appointment; 20% cannot convince clinical staff about their own history because it does not appear on the system [5]. NHS England's own health and care passport guidance formally encourages patients to carry their own continuity summaries [1]. In a single year, 9.1% of all acute presentations — over 11 million occasions — occurred where the treating clinician had no access to the patient's prior record [6].

Three confirmed deaths — the consequence layer:

A three-year-old child with Down's Syndrome died after the NHS 111 adviser had no access to his diagnosis — information that would have triggered immediate clinical assessment [3]. An eleven-year-old died when incompatible IT systems forced verbal-only clinical handover

between ambulance and A&E; critical information was lost in the transfer [3]. A mental health patient was discharged rather than detained because her digital record was unavailable to the receiving team; she died the following morning [3]. Coroners have issued repeated Prevention of Future Deaths warnings to the same organisations without producing structural correction — which is itself evidence that the correction loop, not the individual failure, is the problem [3].

SECTION 9 — COMPENSATION AND HIDDEN BURDEN

The NHS primary care system appears to function. Appointments are booked. Prescriptions are issued. Referrals are made. The strongest structural reading is that it functions because a set of informal human carriers is performing the continuity functions the architecture does not formally support [2][5][6].

Primary carrier — the patient

The patient is the de facto memory function of the NHS primary care system [1][5]. They repeat their history at every appointment because the system cannot retrieve it. NHS England has formalised this with the health and care passport — officially encouraging patients to carry what the system has not been built to surface [1]. Healthwatch England's 2025 survey: 26% of patients with record errors must repeat their full medical history at every appointment [5]. When the patient cannot carry this burden — because they are unconscious, acutely unwell, cognitively impaired, or simply unaware of their own clinical history — the system has no structural fallback. The three death cases in Section 8 are what that looks like [3].

Secondary carrier — the carer

Where the patient cannot carry the burden alone, the carer does. They maintain medication logs across multiple prescribers, bring appointment diaries to consultations, and hold context across transitions that the formal record does not reliably bridge [2][5]. HSSIB's 2025 investigation confirmed: where discharge information has not arrived, the GP relies on the patient or carer's verbal history to make care decisions [2]. This labour is unpaid, unrecognised, and structurally load-bearing.

Tertiary carrier — the GP with accumulated knowledge

For patients with a consistent relationship with a named GP, that GP carries the memory function personally — social context not in the record, family history mentioned once, a previous episode coded under a different symptom [4][6]. This accumulated knowledge is clinically valuable, irreplaceable, and structurally precarious. It exists only for patients with relational continuity. It disappears when the GP retires, moves practices, or is unavailable. It does not survive care transitions [4].

Quaternary carrier — the receptionist and care navigator

The receptionist performs informal triage: identifying complex patients, routing returning patients toward known clinicians where possible, flagging concerns to clinical staff [4]. This is

undesigned, uncompensated as clinical work, and structurally load-bearing. It operates entirely outside the clinical record.

What this means structurally: when any carrier layer withdraws — when the carer can no longer maintain the log, when the patient cannot recall their history, when the known GP is unavailable — the structural failure becomes visible as a clinical event [3][5]. The system appears functional because these carriers absorb the ambiguity it produces. Their labour does not appear in any NHS budget, productivity calculation, or reform proposal.

SECTION 10 — GOVERNING DIAGNOSIS

The governing diagnosis restated precisely:

The NHS primary care system's recurring failure is not best explained as a GP capacity problem, a digital investment shortfall, or a workforce crisis. The structural reading — derived from convergent public evidence across the record architecture, the clinical workflow, the consequence layer, and the patient experience pattern [1][2][3][4][5][6] — is this: the NHS has not failed to create records; it has failed to ensure those records are retrievable as usable clinical continuity at the moment a consequential decision must be made. Every digital reform has addressed this at the wrong layer — improving documentation density and audit visibility without restoring the operational memory function to the point of care. The result is a system that is record-rich and continuity-poor: it generates more data about patients than at any prior point, and less of it arrives in usable form where it is needed. Because no reform has addressed the structural externalisation of the memory function — because the primitive has never been corrected — every downstream fix relocates the same failure: from NPfIT to the SCR, from the SCR to the Shared Care Record, from the Shared Care Record to the NHS App, from the NHS App to a contractual continuity requirement that covers fewer than one in ten patients and has no enforcement mechanism at the record layer [1][2]. The failure is not produced by any individual system, programme, or clinician. It is the predictable output of an architecture that has never been designed to place operational memory at the point of clinical consequence — and has not yet been redesigned to do so.

The primitive mismatch — stated precisely:

	Stated	Actual
Primitive	Joined-up care in which records support direct clinical decisions across the patient's whole NHS journey	Each organisation holds its own records; sharing is request-based or after-the-fact; the patient repeats their history as the default continuity mechanism

What is promised	Continuity of care at the point of need	Audit-grade trace of what happened; operational continuity reconstructed by the patient at each encounter
What is purchased	Clinical judgment supported by retrievable patient history	Clinical judgment formed from whatever the patient can remember, overlaid on a fragmented record
Who succeeds	Patients with relational continuity to a known GP; patients with a carer who maintains logs	Patients who can reconstruct and assert their own clinical history
Who bears the structural cost	Patients who cannot — the unconscious, the elderly, the cognitively impaired, the acutely unwell	The informal carrier layer: patient, carer, known GP, care navigator — unrecognised, uncompensated, structurally load-bearing
What is measured	Appointment volumes, access times, digital adoption rates, documentation compliance	Output metrics that capture documentation compliance without touching the operational continuity layer
What is not measured	Whether usable clinical continuity was present at the point of the consequential decision	The only metric that would reveal the structural failure

Why the mismatch is structural, not incidental:
The mismatch is produced by the collision between forces that have never been reconciled: fragmented NHS IT procurement that created an estate of incompatible EHR systems; organisational record ownership in which each NHS body holds its own records and sharing is an after-the-fact join rather than a native structural feature; and the commercial environment in which GP system vendors control API access and have structural incentives to resist the openness that operational continuity would require [2][6]. These forces have made operational continuity structurally unreachable within the current architecture — not merely difficult, but unreachable without a change at the primitive level.

Validation test:
If NHS England were to implement the Vanish List in Section 12 and structurally require that returning-patient consultations surface retrievable continuity data — including outstanding discharge context and prior unresolved presentations — would the remaining friction pattern be explained by execution quality rather than structural memory absence? If yes, the diagnosis holds. If patients still repeat their history after that structural change is in place, revisit whether vendor API constraints prevent full retrieval — but that would be a constraint on the fix, not a refutation of the diagnosis.

SECTION 11 — COST OF CONTINUATION

The following is a directional assessment of costs generated by the unresolved structural failure. These are derived from public evidence, not internal NHS financial data. Each figure is the financial expression of decisions made without reliable memory at the point of care.

Class 1 — Directly evidenced institutional cost

Clinical negligence [3]
£2.8 billion annually — the cost of diagnostic and referral failures that are the predictable output of authority nodes operating on degraded inputs. 13,784 new claims per year; one every 40 minutes. Failure or delay in diagnosis accounts for 43.5% of all GP indemnity notifications. GP-specific claims rose 9.3% year-on-year.

Preventable deaths [3]
36 Prevention of Future Deaths warnings in 2024 about information sharing failures. The same organisations received repeated warnings without structural correction — confirming that the consequence layer is not generating the redesign it should.

Failed digital reform [2][6]
NPfIT cost over £12 billion [NAO, 2013] without producing a unified continuity record. Each subsequent programme has consumed material resource to produce a partial fix that has not resolved the primitive.

Class 2 — Structurally associated downstream burden

Emergency admissions [6]
A 30% increase in emergency hospital admissions associated with lower GP continuity (Lancet). These admissions are the downstream cost of weak signals that were not surfaced at the point of care.

Acute presentations without record access [6]
9.1% of all acute presentations — over 11 million occasions per year — where the treating clinician had no access to the patient's prior record. Each is a consequential decision made on incomplete information.

Workforce exit [4]
Over 40% of GPs doubt they will still be practising in five years. The Health Foundation projects a one-in-two GP shortfall by 2030/31. As the workforce shrinks, each remaining GP carries a heavier function fusion burden, accelerating exit and eroding the relational continuity that remains.

Class 3 — Invisible patient and carer labour

Patient-carried memory function [1][5]
26% of patients with record errors repeat their full medical history at every appointment. This is unpaid clinical labour — the invisible input that keeps the system appearing to function — borne disproportionately by those with multiple long-term conditions and frequent care transitions.

Carer-carried continuity [2][5]
Where patients cannot carry the burden, carers absorb it. Their labour keeps the handoff chain intact. When it stops, the harm is immediate and documented [3].

Cost of continuation — summary:

Cost category	Annual scale	Source
Total clinical negligence	£2.8 billion	NHS Resolution 2023/24
GP indemnity claims	2,382 new claims; +9.3% year-on-year	NHS Resolution CNSGP 2023/24
Failure / delay in diagnosis (GP)	43.5% of all GP indemnity notifications	NHS Resolution CNSGP
Emergency admissions from continuity failure	+30% associated with low GP continuity	Lancet, 2023
Preventable deaths — information sharing failure	36 coroner PFD warnings, 2024	Coroner records
Acute presentations without prior record access	9.1%; 11 million occasions/year	PMC EHR analysis, 2024
Patients repeating full history at every appointment	26% of patients with record errors	Healthwatch England, 2025
GP workforce exit	40%+ likely to leave within 5 years; projected 1-in-2 shortfall by 2030/31	RCGP; Health Foundation
Failed digital reform — NPfIT alone	£12 billion+	NAO, 2013

The compounding dynamic: workforce decline increases function fusion per consultation → more clinical risk → more negligence claims → more regulatory and administrative burden → more workforce exit. This loop has no internal exit without structural change at the level of the memory function.

Structural upside if corrected: a continuity architecture that places retrievable operational memory at the point of clinical consequence would reduce negligence claims in the

failure-to-diagnose and failure-to-refer categories, reduce emergency admissions, reduce GP administrative burden, and eliminate the structural justification for patient-carried continuity. Primitive correction should be understood as a recovery mechanism, not a discretionary improvement initiative.

SECTION 12 — CORRECTION DEPENDENCY ORDER

This is a dependency order, not a redesign specification. Right fixes in the wrong sequence still fail.

Vanish List — what stops before correction can hold:

Stop	Why
Measuring digital programme success by adoption metrics without measuring continuity retrieval at point of clinical decision [1][2]	Adoption metrics reward the audit layer. The operational layer remains unaddressed.
Treating discharge summary delays as a communication problem [2]	HSSIB has confirmed it is a structural handoff failure. More guidance without enforcement has not resolved it.
Treating ARRS workforce expansion as a continuity improvement [1][4]	Every additional clinician type in the pooled appointment pool further fragments relational continuity. Capacity and continuity are not the same metric.
Treating the NHS health and care passport as a patient empowerment feature [1][5]	It is a formal acknowledgment that the system cannot surface its own operational memory. Name it correctly before designing around it.
Layering digital access features onto the existing fragmented record architecture [1][2]	Each new layer adds documentation trace without restoring retrieval. Scale compounds the mismatch.

Move 1 — prerequisite for everything else:
Name operational continuity as a structural requirement, not an aspiration. NHS England must define what operational continuity means structurally: which actor holds the patient's continuity at the point of care, under what conditions it is retrievable, who has authority to update it, and

what is the minimum standard for its presence when a consequential clinical decision must be made. Without this declaration, every subsequent reform will be interpreted through the existing frame and will produce the same partial fix.

Move 2 — follows from Move 1:
Separate the audit record from the operational continuity record at the data architecture level. The audit record is produced for compliance and defensibility. The operational continuity record must be produced for clinical use — retrievable at the next point of care, enabling the next clinician to act without reconstruction. These require different structures, different custody arrangements, and different incentives. The technical separation is in the full Redesign Artifact.

Move 3 — follows from Move 2:
Redesign the discharge handoff to guarantee continuity transfer before the patient returns. Continuity must be structurally transferred in retrievable form before the patient's follow-up appointment — not left as a delayed communication. The handoff mechanism, format standard, and enforcement consequence are in the full Redesign Artifact.

Move 4 — follows from Move 3:
Mandate a GP IT standard to surface repeat unresolved presentations. When a patient presents with an unresolved clinical picture that has appeared in previous consultations — regardless of which clinician saw them — the system must surface that pattern before the consultation begins. No such standard currently exists. The specification is in the full Redesign Artifact.

Dependency summary: Move 4 cannot precede Move 3. Move 3 cannot precede Move 2. Move 2 cannot precede Move 1. The current pattern of digital programme layering fails precisely because it attempts Move 3 and Move 4 interventions without Move 1 in place. That is why every prior programme has produced partial improvement and then stalled or been replaced.

SECTION 13 — WHAT THIS IMPLIES FOR CORRECTION

This is not a workforce crisis. It is a memory crisis. The governing diagnosis points to a specific class of first priorities — not a full implementation plan, which belongs in the Redesign Artifact.

For NHS England leadership and DHSC: the primitive must be named before any redesign can hold. Pause any new digital access programme that does not first define how continuity retrieval will be measured — not adoption, but retrieval. Communicate honestly that the current architecture externalises memory onto the patient. This is structural, not a failure of individual clinicians or practices.

For NHS Digital and digital transformation: expose discharge summary status to GPs in real time — a low-effort, high structural-impact change that directly addresses the most documented patient harm pathway. Separate the audit record from the operational continuity record at the data architecture level. Do not launch further patient-facing record access features without addressing the underlying interoperability constraints that make continuity retrieval structurally impossible.

For GP practices and clinical safety: identify patients in your list who are carrying the highest continuity burden — repeat presenters, post-discharge patients, those with multiple long-term conditions. A practice-level continuity protocol should define what information must be surfaced when those patients present, regardless of which clinician they see. The informal continuity work your receptionists and care navigators are already performing is load-bearing. It is not supplementary.

Full builder handoff, transition sequence, and system object model are contained in the full Redesign Artifact — available on request.

SECTION 14 — PUBLIC EXEMPLAR NOTE

This document is a public exemplar derived from a full internal diagnostic artifact.

It is shared to demonstrate method, structural capability, and the distinction between diagnosis and redesign. The full internal artifact contains the deeper correction bridge, detailed transition logic, and builder handoff structure not disclosed here.

Full redesign specifications are not publicly available.

SECTION 15 — APPENDIX: EVIDENCE REFERENCES

Evidence tier key:
[1] First-party NHS official sources — stated promise, official workflow, official architecture claims
[2] Independent structural oversight — HSSIB, NAO, Nuffield Trust, RCGP structural findings
[3] Consequence evidence — NHS Resolution, coroner PFDs, confirmed patient harm
[4] Workforce and collapse node evidence — RCGP survey, BMA analysis, BJGP workload studies
[5] Ghost structure and patient experience evidence — Healthwatch England, patient testimony
[6] Academic proof layer — Lancet, BJGP, PMC, Oxford NDPCHS structural and outcomes research

The following is a representative selection. Full source listing is in the master artifact.

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[1] First-party NHS official sources (selected)

- NHS England: Shared Care Records programme documentation and closure (March 2026); transition to Connecting Care Records
- NHS England: Summary Care Record — specification, coverage, and access conditions
- NHS England: NHS App — patient record access guidance
- NHS England: Health and care passport guidance
- NHS England: 2024/25 GP Contract and continuity requirement

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[2] Independent structural oversight (selected)

- HSSIB: Continuity of care — delayed diagnosis in GP practices (2023). No GP contract requirement for continuity; no IT standard for surfacing repeat unresolved presentations.
- HSSIB: Primary and community care coordination for people with long-term conditions (2025). GP relies on patient/carer verbal account when discharge information absent.
- HSSIB: Electronic communications on patient discharge from acute hospitals (2025). Discharge summaries delayed, incomplete, or missed; patient harm documented.
- NAO: Digital transformation in the NHS. Records on inconsistent IT systems; interoperability progress mixed.
- RCGP: Continuity of care in modern day general practice (2025).
- BMA: Pressures in general practice data analysis (2024). 6,000+ FTE GP partner loss.
- Public Digital: NHS App and single patient records (2024). Vendor API control; interoperability constraints.

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[3] Consequence evidence (selected)

- NHS Resolution: Clinical Negligence Scheme for General Practice (CNSGP) 2023/24. £2.8bn total; 13,784 new claims; 43.5% failure/delay in diagnosis.
- Coroner Prevention of Future Deaths reports (2024): 36 warnings on information sharing failures; three confirmed death cases.
- Leary et al., SAGE (2021): Thematic analysis of 710 PFD reports; 36 reports where prior warnings had not produced correction.

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[4] Workforce and collapse node evidence (selected)

- RCGP GP Voice Survey (2024). 76% of GPs believe patient safety compromised; 40%+ doubt they will be in practice in five years.
- PMC: NHS Long Term Workforce Plan analysis (2024). Health Foundation projects one-in-two GP shortfall by 2030/31.
- PMC: Great expectations? GP consultations task analysis (2024). Documentation adds 2 minutes 38 seconds to every encounter.

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[5] Ghost structure and patient experience evidence (selected)

- Healthwatch England: The extent and impact of inaccurate NHS patient records (May 2025, n=1,800). 26% must repeat full history at every appointment; 20% cannot convince clinical staff about their own history.
- NHS England: Health and care passport guidance. Institution formally recommending patients carry their own continuity summary.

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[6] Academic proof layer (selected)

- Lancet Primary Care: Continuity of care in general practice and patient outcomes (2025). Lower mortality and fewer unplanned hospital contacts associated with higher continuity.
- PMC: A call to reconsider a nationwide EHR system (2024). 9.1% of all acute presentations without prior record access; 11 million occasions in one year.
- Oxford NDPCHS: The human connection — why relational continuity matters in general practice (2025).

This public diagnostic exemplar was produced by Jamie Forrester, Independent Systems Architect, Edinburgh, UK. It is based entirely on publicly available information and was produced independently — it was not commissioned by the NHS, NHS England, DHSC, or any associated body. The diagnostic logic, structural interpretation, and governing diagnosis are the author's own.

This public exemplar is shared to demonstrate method and structural capability. Full internal artifacts and redesign specifications are not publicly available.

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